ROXIM CAPSULE 10MG

Description: Sz 2: maroon opaque cap/ maroon opaque body

ROXIM CAPSULE 20MG

Description: Sz 2: maroon opaque cap/powder blue opaque body

Content

Each capsule contains piroxicam l0mg
Each capsule contains piroxicam 20mg

Indications

FOR THE SYMPTOMATIC RELIEF OF PAIN AND INFLAMMATION IN PATIENTS WITH OSTEOARTHRITIS, RHEUMATOID ARTHRITIS AND ANKYLOSING SPONDYLITIS.
HOWEVER IT SHOULD NOT BE THE FIRST CHOICE OF NON-STEROIDAL ANTI-INFLAMMATORY DRUG (NSAID) TREATMENT IN THESE CONDITIONS.

Pharmacology

Non-steroidal anti-inflammatory analgesics (NSAIAs) inhibit the activity of the enzyme cyclo-oxygenase, resulting in decreased formation of precursors of prostaglandins and thromboxanes from arachidonic acid. Although the resultant decrease in prostaglandin synthesis and activity in various tissues may be responsible for many of the therapeutic and adverse effects of NSAIAs, other actions may also contribute significantly to the therapeutic effects of these medications.

Piroxicam is absorbed from the gastrointestinal tract. It is metabolised in the liver by hydroxylation and conjugation with glucuronic acid. Piroxicam is extensively bound to plasma protein (about 99%) and has a long plasma half life of about 35-45 hours. It is excreted in the urine. Less than 5% of the dose is excreted unchanged.

Adverse effects

Gastrointestinal disturbances are the commonest side effects, occuring with piroxicam. Peptic ulceration and gastrointestinal bleeding have been reported. Other side effects which have been reported include headache with dizziness, blurred vision, tinnitus, skin rashes and pruritis and oedema.

Drug interactions

Non-steroidal and-inflammatory drugs may cause sodium, potassium and fluid retention, and may interfere with the natriuretic action of diuretic agents. These properties should be kept in mind when treating patients with compromised cardiac function or hypertension since they may be responsible for a worsening of those conditions.
As with other non-steroidal anti-inflammatory drugs, bleeding has been reported rarely when piroxicam has been administered to patients on coumarm-type anticoagulants. Patients should be monitored closely if piroxicam and oral anticoagulants are administered together.
As with other non-steroidal anti-inflammatory, the use of piroxicam in conjunction with aspirin or the concomitant use of two non-steroidal anti-inflammatory drugs is not recommended because data are inadequate to demonstrate that the combination produces greater improvement than that achieved with the drug alone and the potential for adverse reactions is increased.
Piroxicam is highly protein-bound, and therefore might be expected to displace other drugs. The physician should closely monitor patients for change in dosage requirements when administering piroxicam to patients on higly protein-bound drugs. Non-steroidal anti-inflammatory drugs, have been reported to increase steady state plasma lithium levels. It is recommended that these levels are monitored when initiating, adjusting and discontinuing piroxicam.

Dosage and Administration

Rheumatoid arthritis, osteoarthritis & ankylosing spondylitis : 20mg as single daily dose.
After assessing the risk/benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used.

Precaution

Piroxicam should not be used in patients with a history of gastrointestinal haemorrhage or ulcers or aspirin sensitivity. It should be used with caution in patients with impaired renal function and in patients with cardiovascular disorders where oedema may worsen the condition.

Concomitant administration of piroxicam and aspirin has been reported to result in lowered plasma-piroxicam concentrations.

Drug administration should be closely supervised in patients with a history of upper gastrointestinal disease. It should be withdrawn if peptic ulceration or gastrointestinal bleeding occurs.
Long term administration of piroxicam has resulted in acute interstitial nephritis with hematuria, proteinuria , and occasionally, nephrotic syndrome. Patients at greater risk of such reaction are those with congestive heart (allure, liver cirrhosis, nephrotic syndrome, overt renal disease and those taking diuretics and the elderly. Such patients should he carefully monitored whilst receiving non-steroidal anti-inflammatory drug therapy. Because of report of adverse eye findings with non-steroidal anti-inflammatory drugs, it is recommended that patients who develop visual complaints during treatment with piroxicam have opthalmic evaluation.

- TREATMENT SHOULD ALWAYS BE INITIATED BY A PHYSICIAN EXPERIENCED IN THE TREATMENT OF RHEUMATIC ARTHRITIS.
- USE THE LOWEST DOSE (NO MORE THAN 20MG PER DAY) AND FOR THE SHORTEST DURATION POSSIBLE. TREATMENT SHOULD BE REVIEWED AFTER 14 DAYS.
- ALWAYS CONSIDER PRESCRIBING A GASTROPROTECTIVE AGENT

CardiovascularThrombotic Events

Observational studies have indicated that non-selective NSAIDs may be associated with an increased risk of serious cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease or cardiovascular risk of an adverse cardiovascular event in patient taking NSAID, especially in those with cardiovascular risk factors, the lowest effective dose should be used for the shortest possible duration.

There is no consistent evidence that the concurrent use of aspirin mitigates the possible increased risk of serious cardiovascular thrombotic events associated with NSAID use.

Hypertension

NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking antihypertensive with NSAIDs may have an impaired anti-hypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAID treatment and at regular intervals thereafter.

Heart Failure

Fluid retention and oedema have been observed in some patients taking NSAIDs, there caution is advised in patients with fluid retention or heart failure.'

Gastrointestinal Events

All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulcerations occur in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patient about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of aspirin and NSAIDs also increases the risk of serious gastrointestinal adverse events.

Severe Skin Reactions

NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Stevens-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reactions and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.

WARNING
RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID

Serious GI loxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (eg. dyspepsia are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDS even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (eg. alcoholism, smoking, corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events,

Toxic effects

The reported incidences of adverse effects in patients who take piroxicam is about 20% approximately 5% of patients stop using the drug because of side effects. Gastrointestinal reactions are the most common, the incidence of peptic ulcer is less than 1%, As with other aspirin like drugs, piroxicam alters the function of platelets. and it should be assumed that piroxicam will precipitate bronchoconstriction in patients who are hypersensitive to aspirin.

Contraindication

It is contraindicated in patients with active peptic ulceration, hyper sensitivity to aspirin and NSAID.

PIROXICAM SHOULD NOT BE PRESCRIBED TO PATIENTS WHO ARE MORE LIKELY TO DEVELOP SIDE EFFECTS, SUCH AS THOSE WITH HISTORY OF GASTROINTESTINAL DISORDERS ASSOCIATED WITH BLEEDING OR THOSE WHO HAVE HAD SKIN REACTIONS TO OTHER MEDICINES
PIROXICAM SHOULD NOT BE PRESCRIBED IN ASSOCIATION WITH ANY OTHER NSAID OR AN ANTICOAGULANT.

Symptoms mid Treatment for overdosage and antidote

Gastrointestinal disturbances:
a. Emptying the stomach via induction of emesis or gastric lavage.
b. Administration activated charcoal.
c. Antacids may also relieve gastrointestinal effects.

Storage conditions	:	Keep container tightly closed. Store in a cool, dry place below 30°C. Protect from light. Keep away from children.
Shelf life	:	3 years
Presentation/ Packing	:	1000's
Manufactured & Distributed by	:	**PRIME PHARMACEUTICAL SDN. BHD.** 1505, Lrg Perusahaan Utama 1, Taman Perindustrian Bukit Tengah, 14000 Bukit Mertajam, Penang, Malaysia.

06/09/2018